

DATE: 2 April 2026

TO:

- The Principal Officer, Government Employees Medical Scheme (GEMS)
- Chairperson, GEMS Board of Trustees
- Chairperson, GEMS Risk & Audit Committee
- Chairperson, GEMS Clinical Governance Committee

BY EMAIL AND REGISTERED POST

SUBJECT: PROTECTED DISCLOSURE — SYSTEMIC CLINICAL RISK AND
GOVERNANCE FAILURE — MEDSCHEME / GEMS

Dear Principal Officer and Members of the Board,

This correspondence constitutes a formal Protected Disclosure in terms of the **Protected Disclosures Act (Act 26 of 2000)**. The discloser is a Registered Nurse Specialist and forensic governance practitioner who served as Case Manager within the GEMS Active Disease Risk Management Unit.

The purpose of this disclosure is to formally report a Priority 1 systemic failure within Medscheme-administered systems that presents a material and ongoing risk to GEMS beneficiaries, and to raise concerns regarding the active suppression of this risk at operational and governance level.

GEMS, as the principal medical scheme, bears ultimate fiduciary and statutory responsibility for the conduct of its administrator under the **Medical Schemes Act 131 of 1998** and applicable Council for Medical Schemes (CMS) directives. This disclosure is submitted directly to the Board given evidence suggesting that the matter has not been transparently escalated through administrator channels.

1. Nature of the Systemic Risk

In October 2025, a critical integration failure was identified and formally escalated between:

- The Gexus claims processing system; and
- The Disease Management System (DMS)

This structural failure resulted in:

- Automated non-adherence communications being issued to clinically compliant members
- Potential misclassification of patient adherence status in clinical records
- Risk of inappropriate clinical decision-making by treating practitioners acting on incorrect data
- Member distress, particularly among elderly and vulnerable populations

A Root Cause Analysis (RCA) was conducted internally. The RCA confirmed the integration gap is structural in nature — not an isolated or incidental error. The system failure was, and to the discloser's knowledge remains, unresolved.

2. Confirmed Member Impact

A confirmed case involved a 75-year-old GEMS beneficiary who received an incorrect non-adherence communication despite being fully compliant with prescribed treatment.

The member's daughter communicated to Medscheme in writing:

"I request that you get your house in order and update your records. Stop confusing old lady."

This case demonstrates:

- Failure of data integrity controls prior to member communication
 - Failure of verification processes to prevent incorrect clinical classification
 - Potential reputational, regulatory and legal exposure for GEMS as the principal scheme
 - Possible breach of the Medical Schemes Act's member protection obligations
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3. Internal Escalation and Subsequent Conduct

The matter was escalated internally with supporting documentary and system evidence. Following confirmation of the integration gap:

- The discloser was instructed to contact the affected member and "gently probe" non-adherence — despite evidence that the system-generated communication was incorrect and the integration failure remained unresolved
- The discloser declined on clinical and ethical grounds, documenting the refusal in writing
- Following this ethical refusal, the discloser experienced retaliatory conduct including insubordination allegations
- The system failure was not transparently resolved at operational level and no confirmed remediation plan was communicated

Of particular gravity: an email from the Chief Risk Officer (Monwabisi Kula) dated 18 December 2025 is available to investigators and constitutes evidence of the active suppression of a Priority 1 Material Risk Report at operational governance level. This email was sent following formal submission of a 38-attachment risk escalation on 9 December 2025.

4. Governance and Oversight Concern

The core governance concern underpinning this disclosure is:

A clinically significant, structurally-confirmed system failure affecting GEMS member communications and clinical care may not have been escalated to Scheme-level oversight structures, and may have been actively suppressed at administrator level.

If accurate, this raises material concerns regarding:

- Clinical governance obligations under the Medical Schemes Act 131 of 1998
- Data integrity and member protection standards
- Risk escalation protocols within Medscheme's governance framework
- Administrator accountability to the Scheme under the GEMS-Medscheme service level agreement
- Compliance with Council for Medical Schemes (CMS) regulatory requirements
- Potential breach of the Protection of Personal Information Act (POPIA) in relation to member data integrity

5. Supporting Evidence

Evidence Item	What It Establishes
Documented Root Cause Analysis (RCA)	Structural integration failure confirmed — not isolated
Member communication records	Incorrect non-adherence classification demonstrated
Internal escalation correspondence	Escalation occurred; no remediation plan confirmed
Recorded meeting transcript (post-escalation)	Post-disclosure conduct and instruction to contact member recorded
CRO suppression email — 18 December 2025 (Monwabisi Kula)	Wilful suppression of Priority 1 Material Risk at operational level

All evidence items can be made available to an independent investigator appointed by the Board or by a regulatory authority.

6. Requested Actions

I respectfully request that the Board of Trustees:

1. Commission an independent investigation into the Gexus–DMS integration failure
2. Obtain and review the Root Cause Analysis conducted by Medscheme, and verify whether a remediation plan was implemented
3. Assess the full extent of member impact, including identification of all potentially affected beneficiaries
4. Review the administrator's risk escalation and reporting protocols to the Scheme under the Service Level Agreement
5. Review the conduct surrounding the CRO email of 18 December 2025 and determine whether risk suppression occurred
6. Implement corrective controls to prevent recurrence and report findings to the CMS
7. Confirm in writing whether this matter has previously been disclosed to the Board

7. Regulatory Escalation Notice

This disclosure is made in good faith, under the full protections of the Protected Disclosures Act 26 of 2000.

Should no written acknowledgement of receipt be received within **7 calendar days**, and no substantive response within **14 calendar days** of this date, I reserve the right to escalate this matter without further notice to:

- The Council for Medical Schemes (CMS)
- The Financial Sector Conduct Authority (FSCA)
- The B-BBEE Commission (parallel investigation already underway — Case No. 5/02/2026)
- The South African Human Rights Commission (SAHRC)
- The Parliamentary Portfolio Committee on Health
- The Public Protector
- Relevant media and public interest platforms

A supporting testimony providing full context — including evidence of retaliation, unlawful financial deductions, and systemic personal consequences — is attached as a separate confidential document for the Board's information.

Yours faithfully,

Wilbur William Matthee

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Former Case Manager — GEMS Active Disease Risk Management Unit
Former Acting Chairperson, Health and Safety Committee
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